DIGITAL EDITION | 2026-27

The Indian Special-Needs Child Playbook

From the first worry to the UDID card, free therapy, school rights, scholarships and the Rs. 1 lakh cover

Read this first

This book is a map of the Indian system that surrounds a child with a developmental disability: the free screening that most families never hear about, the disability certificate that unlocks everything else, the therapy that can be had at no cost in a district hospital, the insurance worth Rs. 1 lakh that costs Rs. 500, the deduction worth Rs. 1.25 lakh, the scholarship that pays tuition, and the school that is legally required to keep your child in the classroom.

It is written for the parent who has just been told something and is sitting in a corridor with a file folder, and for the parent three years in who is still paying for everything out of pocket because nobody mentioned the rest of it.

Who this is for

Parents and guardians of children with autism, ADHD, intellectual disability, cerebral palsy, specific learning disability (dyslexia, dysgraphia, dyscalculia), global developmental delay, speech and language delay, hearing impairment, Down syndrome, or any of the 21 disabilities listed in the Rights of Persons with Disabilities Act, 2016. It is also for the aunt, the teacher and the family friend who ends up doing the running around.

The four things that decide how the next ten years go

1. **A written diagnosis.** Not a WhatsApp opinion. A typed, stamped report naming the condition, the tool used and the doctor's registration number. Without it, every other door stays shut.
2. **A UDID card.** The Unique Disability ID is the single document that proves disability to every department in the country. It is free. It is also the slowest thing in this book.
3. **A therapy plan you can actually afford for three years.** Not the best plan in the city. The one that survives month eighteen, when the savings are gone.
4. **A school that follows the law.** Section 16 of the RPwD Act says admit without discrimination. Section 17 lists the accommodations. Most schools know this. Some hope you do not.

What this book is not

- It is not medical advice, and it does not diagnose or treat anything. Therapy decisions belong to your child's own clinicians.
- It is not legal advice. It tells you what the law says and where to complain; for a dispute in court, you need an advocate.
- It is not a guarantee. Officers can sit on a file. A scheme can be revised. Every amount in this book is the amount published by the department at the time of writing, and every one of them should be confirmed at the counter before you plan around it.

How to use it

If you are at the very beginning, read Parts 1, 2 and 3 in order - that is the first six months of your life. If you already have a diagnosis, go straight to Part 2 and get the UDID moving, because it takes months and everything else waits behind it. If the fight right now is school, Part 5 has the sections, the circulars and the letters. Part 4 is money. Part 6 is the part nobody wants to think about, which is why it is the part that hurts most when it is left undone.

A note on words

The law says "person with disability" and "benchmark disability". Families say special needs, neurodivergent, divyangjan, or simply their child's name. This book uses the legal words where a form or an officer will use them, because that is where the words do work, and plain words everywhere else.

Facts, amounts, section numbers and portal names were checked against official sources in November 2026. The sources are listed at the back so you can verify anything before you rely on it. Where a figure is a reported cost rather than a government rate, the book says so.

Quick start: five things you can do this week

1. Write the four-week behaviour diary. One line per incident, five columns. It costs nothing and it is the most useful page you will own.
2. Call the district hospital and ask two questions: does the DEIC see children like mine, and does the medical board have a child psychiatrist or developmental paediatrician?
3. Book the assessment, and ask on the phone whether the report will name the tool, the score and the doctor's registration number.
4. Start the ring file with ten dividers, and scan every paper you already have into one folder on your phone.
5. Write one paragraph to your child's school asking for a meeting, before you ask for anything in it. The letter in chapter 22 is the second step, not the first.

What this book will not do

It will not make an officer move faster, and it will not make a school kinder. What it will do is remove the two reasons people say no: that nobody told you the scheme existed, and that you did not ask in writing. Both of those are fixable this month, and both are free.

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The first ninety days

From the fi
notebook th

CHAPTER 1

The free screening route most parents never use

India runs a national child screening programme that will look at your child's development for free, and almost nobody tells parents it exists. It is called Rashtriya Bal Swasthya Karyakram, RBSK, and its 2026 operating guidelines cover every child from birth to eighteen.

Under RBSK 2.0, mobile health teams screen children at three places. Newborns are screened at the delivery point before discharge. Children from six weeks to six years are screened at Anganwadi centres, at least twice a year. Children from six to eighteen in government and government-aided schools are screened once every academic year. Anything that looks like a developmental delay is referred onward to a District Early Intervention Centre, the DEIC, which is supposed to exist in every district.

Where	Age	How often	What happens next
Delivery point (public hospital)	Birth to 6 weeks	Once, before discharge	Newborn screening for hearing, vision, heart and birth defects
Anganwadi centre	6 weeks to 6 years	At least twice a year	Referral card issued; child sent to the DEIC
Government and government-aided school	6 to 18 years	Once each academic year	Referral card issued; child sent to the DEIC

The list of conditions the programme screens for is not vague. It includes autism and autism spectrum disorder, specific learning disorders, ADHD, global developmental delay, motor delay, cognitive delay, language delay, hearing impairment, vision impairment, cerebral palsy and neuro-motor impairment. That is the 2026 guideline's own list, not a summary.

The part that matters for your family

The DEIC is not a screening desk. It is a multidisciplinary clinic, and the guidelines say it is staffed by paediatricians, psychologists, speech therapists, audiologists, optometrists, physiotherapists, occupational therapists, dentists and social workers. It says services at DEICs are to be provided free of cost to eligible beneficiaries, and that a child identified through screening is referred with a three-part referral card that documents what was observed and what should happen next.

Read that again, slowly, because it is the single most useful paragraph in this book. A district-level team of exactly the specialists you have been paying Rs. 800 an hour for, in one building, at no charge.

What to do if your child is in a private school

The mobile health teams screen government and government-aided schools. That does not lock you out of the DEIC. In practice, parents walk into the DEIC at the district hospital with the child and the reports and ask for an assessment. Some districts accept self-referrals; some ask for a referral from a government doctor, which you can get from the paediatric OPD. Ask for the DEIC coordinator by name, not "the department".

Say this at the counter

"My child is four and has a speech delay. I want a developmental assessment at the District Early Intervention Centre under RBSK. Who is the DEIC coordinator, and what documents should I bring?" Then write down the name of the person who answered. That name is worth more than any phone number.

Two honest limits. First, screening is a net, not a diagnosis: a screen can miss a bright child with subtle difficulties, and it can flag a child who is simply late and catches up. Second, DEIC quality varies by district - some are excellent, some are one overworked therapist and a weighing scale. Go, see what is there, and keep your private assessment going in parallel if you can afford it.

CHAPTER 2

What a proper assessment looks like, and what it costs

An assessment is not a five-minute conversation. For a young child it is usually a ninety-minute session with a structured history, a play-based observation, a standardised tool, and a written report. The clinician should speak to you about what the child does at home, not only what the child does in the room.

Who does what

Professional	What they are for	What you leave with
Developmental paediatrician	Overall development, medical causes, coordinating the plan	A diagnosis or a working diagnosis, plus referrals
Child psychiatrist	Behaviour, autism diagnosis, medication if needed	A diagnostic report with severity
Clinical or rehabilitation psychologist	Standardised testing: cognitive, adaptive, autism severity	Scores, percentiles, a written psychological report
Audiologist (ENT)	Hearing loss, which can look exactly like a speech delay	BERA / OAE report with decibel levels
Speech and language therapist	Speech, language, feeding, communication	A therapy plan with frequency and goals
Occupational therapist	Sensory needs, motor skills, daily living, attention	A sensory profile and a home programme

What the report must contain

- The diagnosis in the words a form will recognise, for example "Autism Spectrum Disorder" rather than "autistic features".
- The tool used and the score, for example an INDT-ASD assessment or an ISAA score. This matters later, because the disability board uses named tools and will ask.
- Severity or level, and any cognitive or adaptive scores.
- Recommendations with frequency: "speech therapy twice weekly for six months", not "therapy advised".
- The doctor's name, qualification, registration number, signature and the clinic's stamp.

What it costs in 2026

Reported figures are wide, and they should be, because a tier-1 metro and a district town are different countries. A parent interviewed in 2026 recalled paying Rs. 2,000 to Rs. 3,000 for an initial assessment a few years earlier, and said the same visit now costs Rs. 6,000 to Rs. 7,000 in a big city. Therapy sessions are reported at Rs. 500 to Rs. 1,000 in Patna in 2026, and from Rs. 600-800 an hour up to Rs. 4,000 an hour depending on the city and the specialist.

Plan for the assessment to cost less than the therapy that follows it. A single assessment is a one-time hit of a few thousand rupees. Eight sessions a month for three years is the number that changes a family's finances, and Part 3 is about getting that number down without getting worse therapy.

Bring this to the first appointment

- The child's immunisation card and any hospital discharge papers.
- Thirty seconds of video of the behaviour that worries you, filmed without the child noticing.
- A one-page timeline: when the child sat, walked, spoke the first word, and anything that was lost.
- The four-week behaviour diary from the next chapter. This is the thing clinicians thank you for.
- A list of your three biggest questions, written down, because in the room you will forget all of them.

The red flag that should end the consultation

Any clinic that promises a cure for autism, offers a machine, a detox, a special diet package or a supplement course as the main treatment, or refuses to put a diagnosis in writing until you buy a package. There is no blood test that diagnoses autism. Walk out politely and keep your money.

CHAPTER 3

Red flags by age, and the diary that saves you six months

Parents are usually right before the professionals are. The problem is that "something is off" is not a sentence a doctor can act on, so it gets converted into "he will talk when he is ready" and two years disappear. The fix is to write down what you actually see.

The milestones worth checking

Age	What should be happening	What is worth raising
6 months	Turns to sound, laughs, reaches for objects, babbles	No response to sound, very stiff or very floppy body
12 months	Sits, points, waves bye, uses gestures, one or two words	No babbling, no pointing, no waving, does not respond to name
18 months	Walks, several single words, follows simple instructions	No words, no pretend play, loses words he had
24 months	Two-word phrases, points to show you things, plays near other children	No two-word phrases, no joint attention, lines up toys for long stretches
3 years	Short sentences, understood by strangers, plays with others	Echoes rather than answers, no back-and-forth, intense fixations
4 to 5 years	Tells a story, follows two-step instructions, shares	Cannot follow instructions, poor name writing, avoids group activity
6 years and up	Reads, writes, sits through a class, keeps friends	Reading far below class level, extreme frustration with writing, daily meltdowns

Any loss of skills at any age is a red flag on its own, whatever the child's age. So is a child who stops responding to his name after having responded before. Screening can reasonably begin at 18 to 24 months; there is no benefit in waiting for a school to notice.

The four-week behaviour diary

For four weeks, one line per incident, five columns. That is the whole method. Take the page to every appointment, and take photocopies: the paediatrician, the psychologist and the disability board will all ask the same questions, and this page answers them without you having to remember anything under pressure.

Date / time	What happened just before	What the child did	How long	What helped

Two things this diary does that nothing else can. It shows a pattern - the same meltdown every day at 4 p.m. after school is a different problem from random meltdowns, and it is fixable. And it converts your worry into evidence. "He has behaviour issues" gets you a nod. "Eleven incidents in twenty-eight days, eight of them in the twenty minutes after he comes home, each lasting fifteen to forty minutes" gets you an appointment.

Keep one folder, not seventeen

Buy one ring file with ten dividers and label it now: Reports, Prescriptions, Bills, School, Government, Therapy notes, Insurance, Photographs, Correspondence, Blank forms. Every paper goes into the file the day it arrives. Scan each one on your phone into a folder called Child-2026. In three years this file will be the difference between a smooth application and a lost year.

CHAPTER 4

Choosing the professional, and reading what they hand you

The first appointment is the most expensive mistake families make, because a general paediatrician will often say "wait and watch" and a parent will wait two years. Choose by what you need answered, not by who is available on a Sunday.

Five questions to ask on the phone before you book

1. Do you do developmental assessments for children of this age, and how long is the first session?
2. Which standardised tools do you use, and will the tool and score be named in the report?
3. Will I get a typed report with your registration number and stamp, and how many days will it take?
4. If my child needs hearing tests first, do you coordinate that, or should I get it done before I come?
5. What is the fee for the assessment and the fee for the report, separately?

A clinic that cannot answer the second and third questions is a clinic that will hand you a prescription slip, and a prescription slip is not a document any government office will accept.

Reading the report you get

- **"Provisional" or "working diagnosis"** means the clinician is not certain yet. That is honest and usable, but ask what would confirm it and by when, and get the review date in writing.
- **"Autistic features" or "autism-like"** is not a diagnosis. For a disability certificate you need the named condition. Ask for the sentence to be rewritten.
- **A low IQ or DQ score** in a child who could not be tested properly is a testing artefact, not a verdict. Ask whether the score is a valid estimate and whether a re-test in six months is advised.
- **"Advise early intervention"** is not a plan. Ask for frequency, therapist type and a review date.

If a report comes back with none of that, you are allowed to ask for it in writing. "Could you add the tool used, the score and your registration number? I need it for the UDID application" is a sentence that works, because it is true and every clinician in India knows exactly what a UDID application needs.

Three copies, always

The original goes into the file and does not leave the house. A photocopy goes to the school. A second photocopy goes to the hospital for the disability board, and you accept that you may not get it back. A scan goes into the phone and into a cloud folder. Reports get lost, and replacing one means paying for the assessment again.

The UDID card

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CHAPTER 5

What the UDID card actually is

The Unique Disability ID is two things in one document: an identity card for a person with disability, and the disability certificate itself. It is issued under the Department of Empowerment of Persons with Disabilities, applied for on swavlambancard.gov.in, and - this surprises almost everyone - it costs nothing. The official scheme page lists the fee as NIL and says the card is delivered free by speed post to the residential address.

A certificate issued this way is valid across the country. That is not a courtesy; it is Section 58(3) of the RPwD Act, 2016. The ministry has also linked UDID cards and disability certificates to DigiLocker, so the digital copy you download is a real document and not a photograph of one.

Why this card is the master key

Almost every benefit in this book asks for it. The ADIP device scheme asks for a UDID with at least 40 per cent disability. The Niramaya insurance asks for a UDID, or a UDID enrolment number plus a certificate for a child under seven. The post-matric scholarship now asks for a UDID card and Aadhaar. NDFDC's loans ask for a UDID number. School concessions run off the certificate. There is no version of this where you skip the card and do the rest.

The 21 disabilities

The RPwD Act recognises 21 specified disabilities, listed in its Schedule. Your child's condition needs to fall inside this list for a certificate, and it almost certainly does if you are reading this book.

Physical	Intellectual and developmental	Others
Locomotor disability	Intellectual disability	Mental illness
Muscular dystrophy	Specific learning disability	Chronic neurological conditions
Leprosy cured	Autism spectrum disorder	Multiple sclerosis
Dwarfism	Multiple disabilities	Parkinson's disease
Cerebral palsy		Haemophilia
Acid attack victim		Thalassemia
Blindness and low vision		Sickle cell disease
Deaf and hard of hearing		Speech and language disability

Benchmark disability: the 40 per cent line

"Person with benchmark disability" means a person certified as having not less than forty per cent of a specified disability, where the disability is not defined in measurable terms; where it is defined in measurable terms, meeting that definition is enough. Forty per cent is the line that matters, because reservation, scholarships, ADIP, NDFDC loans and the higher tax deduction all sit on the far side of it.

One thing the card will not do: it is not a treatment plan and it is not an automatic pension. State disability pensions are separate schemes with their own eligibility and their own office, and a UDID is usually a document you attach to that application rather than the application itself.

CHAPTER 6

Applying, step by step

The application is genuinely simple. The hard part is the medical assessment and the waiting, so the strategy is to get the paperwork perfect on the first attempt and then to chase like a professional.

1. **Register on the portal.** Go to swavlambancard.gov.in and register as a person with disability. A parent or guardian can fill it in and can apply on behalf of a minor.
2. **Fill the form.** Personal details, disability details, address. Aadhaar details are required.
3. **Upload documents.** A colour photograph, signature or thumb impression, identity and address proof, and medical records. Keep every file small and in JPG or PDF - the portal rejects oversized uploads, and an upload that fails silently is a week lost.
4. **Choose the hospital.** Pick the notified government hospital where the right specialists sit. If your district has more than one, this is the single most consequential choice in the application.
5. **Submit and save the enrolment number.** Download the acknowledgement slip. It carries the medical authority assigned to you. Keep two copies.
6. **Attend the assessment.** Contact the hospital's UDID cell for the date. Do not wait to be called; call them.
7. **Collect the certificate and card.** Download the e-UDID and e-certificate from the portal once generated, and let the plastic card arrive by post.

Documents, and the version of each that gets accepted

Document	What to carry	The trap
Photograph	Colour, plain background, recent, small file size	Studio prints scanned at high resolution are usually too large to upload
Signature or thumb impression	On white paper, dark ink, small file	A photographed signature with a shadow behind it gets rejected
Identity proof	Aadhaar, and the Aadhaar number entered correctly	A single wrong digit stalls the whole application at verification
Address proof	Aadhaar, ration card, electricity bill or rent agreement	The address must match what the speed post will find
Medical records	Typed diagnosis report, prescriptions, test reports	Handwritten slips are the most common reason for a defective application
Existing certificate	Old paper disability certificate, if any	You still apply on the portal; the board decides whether to re-assess

What happens in the assessment room

The medical board is not there to treat your child. It is there to measure. The board is chaired by the Medical Superintendent, Chief Medical Officer or Civil Surgeon, or an equivalent notified authority, and sits with a paediatrician or paediatric neurologist or developmental paediatrician, a psychiatrist or child psychiatrist, and a clinical or rehabilitation psychologist. Where specialists are short, the chairperson may co-opt qualified private practitioners onto the board.

Bring the child, the file, the behaviour diary, and one adult who can answer questions without crying. Bring a snack and a toy. Boards run late. Ask at the end, politely, what was recorded and when the certificate will be generated, and write down the answer.

The seven mistakes that cost months

1. Applying with a handwritten prescription instead of a typed report. 2. Choosing a hospital with no child psychiatrist or developmental paediatrician on the board. 3. Uploading files that are too large. 4. Missing the assessment date and not re-booking. 5. Not carrying the old certificate or the school report. 6. Not asking which disability and what percentage was recorded. 7. Never checking the portal again after submitting.

CHAPTER 7

Autism certification, decoded

Autism is certified differently from most disabilities, and parents who do not know the rules spend years confused about why a child who needs a lot of support has a certificate that says 60 per cent.

The rules come from the central government's assessment guidelines notified on 12 March 2024, which superseded the older notifications. For autism spectrum disorder, the guidelines say two separate things: diagnosis is established using the DSM-5-based, AIIMS-modified INCLEN diagnostic tool for ASD, and the severity and percentage are calculated using the Indian Scale for Assessment of Autism, ISAA.

ISAA score	Severity	Disability recorded
70 to 106	Mild autism	40 to 59 per cent
107 to 153	Moderate autism	60 to 79 per cent
Above 153	Severe autism	As certified by the board

The under-six rule

The guidelines are explicit: all children with autism spectrum disorder under six years of age are assessed and given a disability of 60 to 79 per cent, recorded as moderate autism, and they are re-assessed later on the ISAA scale. For a child certified below six, the certificate is temporary and the child must be reassessed between the ages of six and seven. Between six and eighteen, a certificate above 40 per cent stands until a reassessment at eighteen. A certificate issued after eighteen is permanent.

Why this matters practically. If your three-year-old is certified at 60 per cent, that is not a judgement about your child's future; it is the rule for that age band. And the certificate will be revisited at six or seven, so the ISAA score at that point is the one to prepare for, with a fresh assessment from a psychologist who tests in your child's home language if possible.

What a good autism assessment contains

- The diagnostic tool named, with the date of assessment and the clinician's qualification.

- The ISAA score out of 200, with the domain breakdown if the clinic provides it.
- A note on whether intellectual disability is also present, because a second condition can change the composite percentage.
- The percentage recommended, and the period of validity.

Two honest caveats, because you will meet them. The 2024 guidelines give bands rather than a single number - moderate autism is 60 to 79 per cent - and clinicians have published criticism of exactly this, pointing out that it is unclear whether a board should assign the lower end, the upper end or a point inside the band. Separately, ISAA is known to perform less well in children under five, which is precisely the age at which most families first apply. Neither caveat is a reason to avoid applying. Both are reasons to ask, at the assessment, which number was recorded and why, and to use the appeal route in chapter 9 if the answer does not match the report.

CHAPTER 8

Tracking, chasing, and what to do when nobody calls

The portal is not the bottleneck. The hospital is. Applications sit because a UDID cell has one clerk, three hundred files and no incentive to move, and because the family assumes that waiting is the process.

The delay is real and documented. A report in October 2026 found that it takes an average of 218 days for a disabled person in Delhi to receive the card, placing the capital among the worst-performing states and union territories on this measure. Plan for months. Start in the month you get the diagnosis, not the month you need the card.

Tracking your application

On the portal, choose Track Your Application and search with any one of four identifiers: the enrolment number, the UDID number, the Aadhaar number or the registered mobile number. If you have lost the acknowledgement slip, track first to recover the enrolment number, then log in with the enrolment number and date of birth to download the receipt. The portal also runs an AI-assisted chatbot for status queries and general UDID questions.

The escalation ladder

Step	Who	What you ask for
1	Hospital UDID cell / medical authority	The assessment date, in writing, this week
2	Chief Medical Officer or District Welfare Officer	Why the file is pending, and a date
3	State UDID coordinator	Escalation of a file stuck beyond the state's own timeline
4	District Collector / Magistrate	Chairs the Local Level Committee; a letter from this office moves files
5	State Commissioner for Persons with Disabilities	A written complaint under Sections 79 and 80 of the RPwD Act
6	CPGRAMS	A formal grievance against the department, with your file numbers attached

Two rules for all of it. Everything in writing, even if you also phone. And every letter ends with a specific ask and a date: "I request that my child be given an assessment date before 30 November 2026", not "kindly do the needful".

The phone script that works

"Good morning. My child's UDID enrolment number is _____, applied on _____ at _____ hospital. The application is showing as pending at your office. I am not complaining - I only need the assessment date, because the school admission closes in March and the certificate is needed before that. Can you tell me the next board date, and can I bring the file to you personally this week?"
Then note the date, the name of the person, and call again three days before the promised date.

CHAPTER 9

When the percentage is wrong: the Section 59 appeal

Section 59 of the RPwD Act gives any person aggrieved by a decision of the certifying authority a right of appeal to an appellate authority designated by the state government. In practice, since July 2024, that appeal runs through the UDID portal itself.

1. **File within 90 days.** The portal's standard operating procedure, effective 1 July 2024, allows an appeal within 90 days from the date of issue of the card. After that the option disappears from your dashboard.
2. **Log in and find the button.** Log in to your dashboard with the UDID number and date of birth, and click "apply for review of decision". State the ground of appeal.
3. **Know who will hear it.** The appellate board is set by the state and is usually the same hospital with different board members, another authorised hospital in the same district, or a hospital in a neighbouring district within a cluster.
4. **You get one appeal.** The SOP says the second assessment is final even if it is less favourable than the first, and the software allows the appeal option once. Treat it as a single shot and prepare properly.
5. **If nobody acts for three months, reapply.** The rejection-handling SOP of 29 October 2024 allows a fresh application where the appellate authority has not acted within three months of the appeal.

Grounds that actually work

- The wrong assessment tool was used, or the report you submitted was not considered at all.
- A second condition was never assessed - for example autism was recorded but the hearing loss in the audiology report was ignored, which affects a composite percentage.
- The recorded percentage is inconsistent with the ISAA score in your own report.
- The certificate records the wrong disability type, which quietly disqualifies the child from schemes meant for that disability.

Write the appeal as three short paragraphs: what was applied for and when, what the board recorded, and what the evidence shows instead, with the page numbers of your reports. Attach the reports again. Do not argue about how disabled your child is; argue that the recorded number does not match the documented evidence. That is a question a board can answer.

CHAPTER 10

Your UDID file, and the four mistakes that cost months

A UDID application is not one document. It is a small dossier, and the families who get through it in three months instead of twelve are the ones who assembled the dossier before they applied.

Item	Have it?	Notes
Typed diagnosis report with stamp and registration number		The foundation of everything else
Aadhaar of the child, or the enrolment slip		Required; check every digit twice
Aadhaar and address proof of the parent applying		The address decides which hospital and which post office
Photograph and signature, correctly sized		Small files; test the upload before submitting
Previous prescriptions and therapy notes		Shows duration of the condition, not just its existence
Audiology report, if speech is the concern		Hearing loss mimics autism and changes the plan
School report or teacher's note		Useful for learning disability and ADHD assessments
Behaviour diary, four weeks		The most persuasive single page in the file
Enrolment number and acknowledgement slip		Two photocopies, one in the file, one in the bag
Old paper disability certificate, if any		Attach it; ask for digitisation rather than a fresh assessment

The four mistakes

- **Applying before the report exists.** The portal will accept the application and the hospital will reject it as defective. Get the typed report first.
- **Choosing a hospital for its distance.** A hospital forty kilometres away with a child psychiatrist on the board beats the one at the end of your street with nobody who assesses autism.
- **Not showing up, and not re-booking.** A missed board date is not a small thing; it can push you to the back of a queue measured in months.
- **Forgetting the reassessment.** A certificate issued under six is temporary. A child who turns eighteen with a certificate that needs renewal at eighteen, and a family that finds out at twenty-two during an admission, is a story that repeats every year.

Do this today

Open the portal, create the login, and save the password somewhere your spouse can find it. Then make a phone call to the district hospital and ask one question: "Does your medical board have a child psychiatrist or developmental paediatrician for autism assessments, and what is the next board date?" That single call decides whether the next six months are smooth or not.

Therapy you can actually afford

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CHAPTER 11

The DEIC: free multidisciplinary therapy in your district

The District Early Intervention Centre is the most under-used public service in child disability in India. Under the 2026 RBSK guidelines, DEICs are described as the primary referral and service delivery hubs, providing multidisciplinary therapeutic care to children from birth to eighteen. The guidelines say every child identified through screening is referred to a DEIC for evaluation, confirmation and management, and that all services at DEICs are to be provided free of cost to eligible beneficiaries.

The staffing model in the guideline is the part that should make you sit up: paediatricians, medical officers, psychologists, speech therapists, audiologists, optometrists, physiotherapists, occupational therapists, dentists and social workers, in one team. That is a private multidisciplinary assessment worth tens of thousands of rupees, assembled by the state.

What a DEIC can realistically give you

Usually available	Often limited	Do not expect
Developmental assessment and a written opinion	Autism-specific behavioural therapy (ABA-style)	A guaranteed weekly slot for years
Physiotherapy, occupational therapy, speech therapy sessions	Trained special educators	One-to-one attention for a full hour
Hearing and vision screening, referral for surgery	Parent training programmes	A diagnosis of a condition outside its scope
Referral to tertiary hospitals for complex care	Continuity when staff are transferred	Transport or lost-wage support

How to get in

1. Find your district's DEIC. It is usually at the district hospital or attached to it. Ask the hospital reception for the "DEIC" or "RBSK early intervention" room, and ask for the coordinator by designation.
2. Carry the file: typed report, prescriptions, audiology if done, immunisation card, Aadhaar, and the behaviour diary.
3. Ask for an assessment and a written therapy plan with frequency. Free does not mean informal; a written plan is what lets you hold anyone to anything.
4. Book the next appointment before you leave the building. A DEIC that runs on walk-ins will lose you in the crowd.

The realistic arrangement for most families is a hybrid: DEIC or DDRC for two of the three therapies, one private therapy for the goal that matters most this year, and a home programme that you run daily. That combination costs a fraction of an all-private plan and is not clinically inferior, because therapy dose matters more than therapy brand.

Ask the DEIC for these five things in writing

The developmental diagnosis or working diagnosis. The therapy plan, with frequency and goals. The review date. A referral to the disability board for the UDID, if not already done. A parent-training slot, because the hours you spend with your child outnumber the therapist's by a factor of fifty.

CHAPTER 12

DDRCs, CRCs and the National Institutes

Below and above the DEIC there are two more layers of the same public system, and both are open to you without a referral in most states.

District Disability Rehabilitation Centre

The DDRC is a district-level rehabilitation centre, set up under a central scheme and run by the district administration with a partner agency, with one envisaged in every district. Its own service list reads like a private rehab clinic's brochure: early identification and intervention, physiotherapy, occupational therapy, speech therapy, assessment and fitment of assistive devices, skill training, referral for surgical correction, help with scholarships and self-employment loans, and assistance in issuing the UDID card.

The 2026 position is that DDRCs are established and operated with state support and function under the district magistrate, with the centre mainly providing viability-gap funding. A 2026 parliamentary reply noted that an impact evaluation found DDRCs improving local access and early identification. There are around 120 DDRCs, alongside five Composite Regional Centres and four Regional Rehabilitation Centres, according to a 2026 policy review. Not every district has one yet - ask at the district hospital.

Composite Regional Centres and National Institutes

The National Institutes under the Department of Empowerment of Persons with Disabilities are the apex public bodies for specific disability groups - intellectual disability, multiple disabilities, speech and hearing, physical disability - and the Composite Regional Centres are their regional arms. They run OPD assessments, therapy, early intervention, parent training, special education, vocational training and research, usually at nominal cost.

For a family in a tier-2 or tier-3 town, one visit to a National Institute or CRC is worth the train fare: you get a specialist opinion that your district cannot provide, a written plan your local therapist can follow, and often a parent-training programme you can repeat at home. Ask for the OPD schedule before travelling, and ask whether the centre has a residential or hostel facility for the duration of an assessment.

Layer	What it is for	What it costs
Anganwadi / school screening	Finding children who need a look	Free
DEIC	Multidisciplinary assessment and therapy, birth to 18	Free
DDRC	District rehabilitation, devices, skill training, UDID help	Free or nominal
CRC / National Institute	Specialist opinion, parent training, complex cases	Nominal
Private clinic	Intensity, choice of therapist, scheduling that fits work	Rs. 500 to Rs. 4,000 a session

CHAPTER 13

ADIP: devices, learning kits and a Rs. 7 lakh cochlear implant

The ADIP scheme - Assistance to Persons with Disabilities for Purchase or Fitting of Aids and Appliances - is a central scheme running since 1981 and last revised on 26 September 2024. It pays for the physical things a child needs, and it is far broader than most parents realise: hearing aids, motorised tricycles and wheelchairs, artificial limbs and orthoses, teaching and learning material kits, smartphones and magnifiers, and cochlear implants.

Who qualifies

- An Indian citizen of any age, holding a UDID card or a UDID enrolment number with a disability certificate of at least 40 per cent.
- Aadhaar, or an Aadhaar enrolment number.
- Monthly family income from all sources not exceeding Rs. 30,000. For a dependent child, the income of parents or guardians is what counts.
- No assistance for the same purpose from any source in the last three years - though for children the waiting period is shorter. The scheme's FAQ says one year for children below eighteen; the 2024 scheme text says one year for children below twelve. Confirm which applies at the centre you use.

How much it pays

What	Assistance	Conditions
Aids and appliances up to Rs. 15,000	Full cost up to Rs. 22,500 monthly income; 50 per cent for Rs. 22,501 to Rs. 30,000	Once in three years
Aids costing Rs. 15,001 to Rs. 30,000	Up to Rs. 15,000	Once in three years
High-end prosthesis	Up to Rs. 30,000	40 per cent disability and above
Motorised tricycle or wheelchair	Up to Rs. 50,000	80 per cent disability and above, age 16 and above, once in five years
Cochlear implant with surgery, therapy and mapping	Up to Rs. 7 lakh for pre-lingual hearing loss aged 1 to 5; up to Rs. 6 lakh for acquired loss aged 5 to 18	Nodal agency AYJNISHD Mumbai; device procured by ALIMCO; surgery at empanelled hospitals
Travel and boarding	Rail or bus fare for the person and one escort; boarding of Rs. 100 a day for up to 15 days	For families with income up to Rs. 22,500 a month

The two clauses written for your child

First, teaching and learning material kits. The 2024 scheme text allows a temporary disability certificate for a child up to six, or a developmental delay certificate from a competent medical authority, to be considered for TLM kits for children with intellectual and developmental disabilities, without diluting the 40 per cent rule. That is a door for exactly the family that has a diagnosis but no UDID yet.

Second, ADIP-SSA camps. ADIP funds camps that distribute aids and assistive devices to school-going children under Samagra Shiksha, and in those camps the 80 per cent requirement for

a motorised wheelchair is relaxed to 40 per cent for students.

How to apply

Three routes. Apply online on the ARJUN portal, which is the department's joint interface for ADIP and the Rashtriya Vayoshri Yojana. Walk into a Pradhan Mantri Divyasha Kendra - around 100 are operational across the country - where eligible beneficiaries can get aids and appliances under the scheme. Or go through a DDRC, a Composite Regional Centre, a National Institute, ALIMCO or a state corporation, all of which are implementing agencies.

Bring a prescription that names the device. "Hearing aid advised" is weaker than "bilateral behind-the-ear digital hearing aid, moderate to severe sensorineural loss, right ear 65 dB, left ear 70 dB". The second sentence tells the agency what to fit and why, and it is what a good audiologist will write if you ask.

CHAPTER 14

Building a therapy budget that survives three years

Here is the arithmetic that empties family savings. Three therapies, eight sessions a month each, at Rs. 700 a session, is Rs. 16,800 a month. That is Rs. 2,01,600 a year, and Rs. 6,04,800 over three years, before assessments, travel, devices and school. Reported Indian costs in 2026 range from Rs. 15,000 to Rs. 30,000 a month for therapy alone, and a parent interviewed in 2026 put the total cost of raising a child on the spectrum at Rs. 3 lakh to Rs. 10 lakh a year, depending on the city and the intensity.

That number is not a reason to do less therapy. It is a reason to structure it, because the families who collapse at year two are usually the ones who spent the first year as if the first year were the whole race.

The 40-40-20 structure

- **Forty per cent free.** DEIC and DDRC sessions, which cost nothing and cover physiotherapy, occupational therapy and speech therapy in most districts.
- **Forty per cent subsidised.** Devices through ADIP, therapy reimbursed through Niramaya, the school concessions that replace a paid shadow teacher, and the tax deduction that returns part of what you spent.
- **Twenty per cent private and intensive.** The one or two goals that need a specialist and a schedule that fits your work. Spend here deliberately, not by default.

Monthly cost line	Typical 2026 range	How to reduce it
Speech therapy	Rs. 500 to Rs. 1,000 a session in smaller cities; Rs. 600-800 to Rs. 4,000 an hour in metros	DEIC or DDRC slot for one session a week; group sessions
Occupational therapy	Same band	Home programme written by the OT; review every three months
Special education or shadow teacher	Rs. 10,000 to Rs. 15,000 a month where the school insists	Ask the school for a resource-room placement instead; it is a legal ask
Assessment and review	Rs. 2,000 to Rs. 7,000 per assessment	DEIC assessment; annual rather than quarterly review
Devices and aids	Rs. 3,000 to Rs. 30,000 per item	ADIP, once in three years, at 50 or 100 per cent subsidy
Travel	Rs. 500 to Rs. 3,000 a month	Cluster appointments on one day; Niramaya reimburses up to Rs. 2,000 of transport

Cut cost without cutting dose

1. Ask every therapist for a written home programme and do it daily. Twenty minutes a day, seven days a week, is more therapy hours per month than one weekly clinic session.
2. Move to fortnightly reviews once goals are stable, and use the saved money for the therapy that is actually moving.
3. Set one measurable goal per therapy per quarter - "requests help verbally ten times a day" - and drop or change any therapy that has not moved the goal in two quarters. This is not ruthlessness; it is how good clinicians work.
4. Cluster your appointments on two days a week so one parent can keep working.
5. Use Niramaya's Rs. 20,000 ongoing-therapy sub-limit and Rs. 2,000 transport sub-limit every year, even if the reimbursement is slow. It is your money.

Month	Speech	OT / Physio	Special ed	Travel	Total

Fill this in for one month before you decide anything. Most families discover that a third of the spend is on a therapy nobody can say the goal of.

Three questions that cut a therapy bill without cutting therapy

1. What is the measurable goal of this therapy for the next three months, and how will we know it moved? If nobody can answer, the sessions are supervision, not therapy. 2. Can this be delivered as a home programme with a review every three months, at a third of the cost? For occupational therapy and early language work, the answer is usually yes. 3. Is there a free or subsidised slot for this same therapy within ten kilometres of my house? Ask the DEIC and the DDRC before you assume there is not.

The money

Rs. 1 lakh c
tuition. Loa
same card.

CHAPTER 15

Niramaya: the Rs. 1 lakh cover for Rs. 500

Niramaya is a health insurance scheme run by the National Trust for persons with autism, cerebral palsy, intellectual disability and multiple disabilities. It gives cover of up to Rs. 1 lakh a year, with no pre-insurance medical test, at a single premium across all age bands, and treatment can be taken at any hospital.

What it costs

Category	Enrolment	Renewal	Documents
Below Poverty Line	Rs. 250	Rs. 50	BPL card, UDID, address proof, photograph
Non-BPL	Rs. 500	Rs. 250	Income certificate, UDID, address proof, photograph
Person with a legal guardian other than natural parents	Free	Free	Legal guardian certificate from the Local Level Committee, UDID

Enrolment is open through the year, and the cover runs from the date the health card is issued to 31 March of that financial year. That last point matters: enrol in February and you get two months of cover for the full fee. Enrol in April. Renewal should be filed about three months before expiry.

What the Rs. 1 lakh is made of

Section	What it covers	Sub-limit
I	Hospitalisation, including corrective surgery for an existing disability	Rs. 55,000
I (A)	Corrective surgeries, with a cap of Rs. 15,000 for surgeries other than those for existing disability	Rs. 40,000
I (B)	Non-surgical hospitalisation	Rs. 15,000
II	Out-patient treatment: medicines, pathology, diagnostic tests	Rs. 19,000
II (A)	OPD treatment	Rs. 15,000
II (B)	Preventive dentistry	Rs. 4,000
III	Ongoing therapies to reduce the impact of disability	Rs. 20,000
IV	Alternative medicine (AYUSH)	Rs. 4,000
V	Transportation costs	Rs. 2,000

The rules that decide whether you get paid

- It is reimbursement only. You pay the clinic, then claim. There is no cashless facility.
- The UDID card is mandatory, with the disability covered under the National Trust Act. For a child below seven, a UDID enrolment number with a disability certificate is accepted.

- For ongoing therapies, the claim needs a doctor's prescription at least once in six months, naming the category of therapy and its duration. After that, documents from the rehabilitation professional and the bills are accepted.
- Original reports and films - MRI, X-ray, ultrasound - are required to settle claims. Keep them.
- The scheme covers four disability groups only: autism, cerebral palsy, intellectual disability and multiple disabilities. A child with only a learning disability or only hearing impairment is not covered here.

States add their own money on top. Kerala, for example, pays the Niramaya premium for its residents through the Social Justice Department, with applications collected through schools, Anganwadis, Kudumbasree units and Local Level Committees. Ask your state social welfare department whether it does the same; it is a ten-minute question with a five-figure answer.

Enrol this week

Enrolment is through the National Trust portal, a National Institute or Composite Regional Centre, or a registered organisation of the National Trust. The claims helpline published by the National Trust is 040-68178569 and 040-71326135. Keep the e-card on your phone and a printed copy in the file: the cover date is printed on it, and that date is what the insurer will argue about.

CHAPTER 16

Health insurance when your child already has a diagnosis

Most parents are told that no insurer will cover a child with autism. That is not what the regulator says. On 27 February 2023, IRDAI issued a circular requiring every general insurer and stand-alone health insurer to offer a specific product for persons with disabilities, persons with HIV/AIDS and persons with mental illness, and directing insurers to put in place a board-approved underwriting policy that ensures no proposal from these groups is denied for the reason of the disability or illness.

The circular also sets the floor for the product design: insurers may widen the scope but may not narrow it, the policy tenure is one year and renewable, and pricing follows the health insurance regulations. That sits alongside Section 3 of the RPwD Act, which prohibits discrimination on the ground of disability, and Section 24(3)(j), which provides for a comprehensive insurance scheme for persons with disabilities not covered by specified schemes.

What it looks like in practice, in 2026

The gap between the rule and the counter is real, and it is being litigated. In an order reported in October 2026, the insurance ombudsman examined a complaint from a father who sought a family floater for himself, his wife and two children with disabilities: coverage for the children was initially not offered, then offered with a substantially higher premium, co-payment conditions of 20 to 30 per cent, and separate individual policies instead of the floater he had asked for. The regulator's own submission was that a proposal cannot be rejected solely on the ground of disease, medical condition or disability, while premium and terms are set actuarially. The ombudsman recommended that the insurer place on record the basis for the higher premium and the co-payment.

Separately, in September 2026 the Supreme Court, hearing a public interest petition on the rights of persons with neurodevelopmental disabilities, directed IRDAI to place on record data on the number of policies issued by public and private insurers to persons with autism, cerebral palsy, intellectual disability and multiple disabilities, with the matter listed again in November 2026.

What to do at the counter

1. Ask for the insurer's PwD product by name. Every insurer has to have one; the sales person may not know that, so ask for the product brochure, not for advice.
2. Disclose the diagnosis in full at proposal stage. Non-disclosure is the one thing that lets an insurer void a claim, and it is the trap parents fall into when they are told to "just not mention it".
3. Compare three numbers, not one: premium, co-payment percentage, and the waiting period for the condition. A cheap policy with 30 per cent co-pay is expensive the year you need it.
4. Ask in writing whether the policy is a floater or an individual policy, and why, if the answer is different for your child than for you.
5. If refused, ask for the refusal in writing, complain to the insurer's grievance officer, then escalate to IRDAI's Bima Bharosa portal and, if unresolved, to the insurance ombudsman for your state.

One honest expectation to set: even a good health policy usually covers hospitalisation, not weekly out-patient therapy. That is what Niramaya's OPD and therapy sections exist for, and why you should hold both rather than choosing.

CHAPTER 17

The disability deduction: Rs. 75,000 or Rs. 1,25,000 a year

Two deductions in the income-tax law exist for exactly this situation, and they are flat amounts - you do not need bills for every therapy session, only the certificate.

Old section	New section	For whom	Amount
80DD	127	Maintenance or medical treatment of a dependant with disability	Rs. 75,000; Rs. 1,25,000 for severe disability of 80 per cent or more
80U	154	A taxpayer who is himself or herself a person with disability	Rs. 75,000; Rs. 1,25,000 for severe disability

The Income-tax Act, 2025 took effect on 1 April 2026 and renumbered almost every section; the deductions moved into Chapter VIII. The amounts and conditions were carried over substantially unchanged, but the numbers changed, and different published tables disagree on one or two of them. Before you file, check the mapping on the Income Tax Department's own old-to-new comparison utility rather than trusting any article, including this one.

The five things that decide whether you can claim it

- The deduction is available under the old regime. The new regime, which is the default, does not allow it. If you want it, you have to opt for the old regime and compare the two totals honestly.
- For Section 127, the person with disability must be a dependant, and you must be a resident individual or HUF. You cannot claim both 127 and 154 for the same person.
- A medical certificate from the prescribed authority is required and is furnished with the return.
- A flat deduction means the amount does not depend on what you actually spent. Spend Rs. 20,000 on therapy and the deduction is still Rs. 75,000, if the certificate says so.
- At the 30 per cent slab, Rs. 1,25,000 of deduction is about Rs. 39,000 of tax before cess. That is more than three years of Niramaya premiums.

CHAPTER 18

Scholarships: from Class 9 to a Master's

The Department of Empowerment of Persons with Disabilities runs a central umbrella scheme of scholarships for students with disabilities. The guidelines were revised in September 2024 and the components are applied through the National Scholarship Portal. Every one of them needs a benchmark disability of 40 per cent or more, a UDID card, and Aadhaar-based authentication.

Component	For	What it pays	Income ceiling
Pre-matric	Classes 9 and 10	Maintenance and book support; 25,000 slots across components	Rs. 2.5 lakh
Post-matric	Class 11, 12, diploma, degree, post-graduate	Tuition and admission fee up to Rs. 1.4 lakh; other compulsory fees up to Rs. 10,000; maintenance of Rs. 1,200 a month for hostellers or Rs. 650 for day scholars, for 12 months; disability allowance of Rs. 4,000 for visual impairment or intellectual disability and Rs. 2,000 for other disabilities; books Rs. 1,500	Rs. 2.5 lakh
Top Class Education	Graduate and post-graduate degree or diploma at a notified Institute of Excellence	Tuition up to Rs. 1.90 lakh a year; other fees up to Rs. 10,000; Rs. 45,000 one-time for a laptop; Rs. 30,000 one-time for aids; maintenance of Rs. 3,000 a month for hostellers or Rs. 1,500 for day scholars; special allowance of Rs. 2,000 a month; books Rs. 5,000	Rs. 8 lakh

The rules that quietly reject applications

- Slots are fixed - 17,000 for post-matric and 300 for Top Class, including renewals - and renewals are processed before fresh cases. Applying early is not a formality.
- Only two children with disabilities of the same parents can receive the benefit.
- One class, one year. Repeating a year means no scholarship for that year.
- You cannot hold another scholarship or stipend at the same time.
- A student who has already passed one stage and enrols in the same stage in a different subject is not eligible - a B.Com after a B.A. - though professional courses such as LL.B, B.Ed and B.El.Ed after a degree are allowed.
- Merit is ranked by the percentage of disability on the certificate, and for specific learning disability, where no percentage is written, 40 per cent is used.

For the 2026-27 session the National Scholarship Portal window ran from June and closed on 31 October 2026. If you are reading this after that date, the work to do now is documentary: get the UDID, get the income certificate from the revenue office, keep fee receipts, and set a reminder for late May 2027, because the window opens in June and the institute verification deadlines are unforgiving.

CHAPTER 19

Loans at four to nine per cent

The National Divyangjan Finance and Development Corporation - NDFDC, formerly NHFDC - lends to persons with disabilities at rates a commercial bank will not match, for self-employment, education and housing. The eligibility is short: an Indian citizen with 40 per cent or more disability, a UDID number, and age above 18 (above 14 for a person with intellectual disability). There is no upper age limit and no income ceiling.

Loan	Amount slab	Rate to the borrower
Self-employment	Up to Rs. 50,000	5 per cent
Self-employment	Above Rs. 50,000 to Rs. 5 lakh	6 per cent
Self-employment	Above Rs. 5 lakh to Rs. 15 lakh	7 per cent
Self-employment	Above Rs. 15 lakh to Rs. 30 lakh	8 per cent
Self-employment	Above Rs. 30 lakh to Rs. 50 lakh	9 per cent
Education, in India or abroad	Up to Rs. 50 lakh	4 per cent
Home loan	As per slab	7 per cent and above

A one per cent rebate applies to women with disabilities on self-employment loans up to Rs. 50,000. The upper limit across schemes is Rs. 50 lakh per beneficiary or unit, repayment runs up to ten years, and the money is routed through State Channelizing Agencies nominated by state governments, public sector banks, regional rural banks and partner microfinance institutions. The list of channelizing agencies is on the corporation's website, and that list - not the head office - is where your application starts.

What this is good for, in practice: a tailoring or food unit run by a young adult with a disability, a computer and furniture for a home tuition business, a modified vehicle, or a master's abroad that a bank would price at 11 per cent. What it is not: a grant. It is a loan with interest and repayment, and the agency will assess whether the activity can repay it.

CHAPTER 20

Free coaching, fellowships, and the schemes nobody mentions

Three more central schemes are worth knowing about, because they target the years when a family's costs peak.

Free coaching for competitive exams

The department's free coaching scheme pays the coaching fee itself - up to Rs. 75,000 for civil services, JEE, NEET and GATE, and up to Rs. 40,000 for SSC, RRB, banking, CLAT, CAT, CA-CPT, TET, UGC-NET and CUET - and adds a stipend of Rs. 4,000 a month, a disability allowance of Rs. 2,000 a month and a one-time book allowance of Rs. 5,000. Family income must be within Rs. 8 lakh a year, and there are about 1,000 slots, delivered through the department's National Institutes and Composite Regional Centres with a minimum of sixteen hours of physical coaching a week.

National Fellowship for Persons with Disabilities

For research degrees, the fellowship pays at UGC rates - Rs. 31,000 a month for the first two years on the Junior Research Fellowship pattern and Rs. 35,000 a month for the remaining three on the Senior Research Fellowship pattern - plus contingency, an escort or reader allowance of Rs. 2,000 a month, and house rent allowance at 9, 18 or 27 per cent for those without hostel accommodation. Selection runs off the UGC-NET and UGC-CSIR NET merit list.

Scheme	Who runs it	What you get	Where
Niramaya	National Trust	Rs. 1 lakh health cover, Rs. 500 premium	National Trust portal
ADIP	DEPwD	Devices, kits, implants, up to Rs. 7 lakh	ARJUN portal, PMDK, DDRC
Scholarships	DEPwD	Tuition, maintenance, laptop, aids	scholarships.gov.in
Free coaching	DEPwD institutes	Fees up to Rs. 75,000, stipend	National Institutes / CRCs
NDFDC loans	NDFDC	5-9 per cent self-employment, 4 per cent education	State Channelizing Agency
Disability deduction	Income Tax Department	Rs. 75,000 or Rs. 1,25,000	Your return, old regime
UDID	DEPwD	The card that unlocks the rest	swavlambancard.gov.in

Every one of these is a government page, and every one of them is updated without notice. Before you plan around an amount, open the department's own page and read the current guidelines - not a summary, not a YouTube video, not this book. The scheme pages carry a PDF of the guidelines, and the PDF carries the conditions that decide whether your application survives verification.

How to check a scheme in five minutes

Search the scheme name with the word guidelines and restrict the result to a .gov.in or .nic.in domain. Open the most recent PDF, not the web summary. Look for three things: the eligibility clause, the income ceiling, and the date of revision printed on the first page. If the document is more than three years old, ask the implementing office whether a newer one exists. That five-minute habit is worth more than any list of schemes, because it never goes out of date.

School

The law is
the letter th

CHAPTER 21

What the law actually says about school

Section 16 of the RPwD Act, 2016 is short and unusually concrete. It says the appropriate government and local authorities shall endeavour that all educational institutions funded or recognised by them provide inclusive education to children with disabilities, and towards that end shall do eight specific things. The first is to admit them without discrimination and provide education, sports and recreation opportunities equally with others. The others cover accessible buildings and campuses, reasonable accommodation according to the individual's requirements, individualised support, appropriate modes of communication, early detection of specific learning disabilities, monitoring of participation and progress, and transport facilities for children with disabilities and the attendant of a child with high support needs.

Section 17 then lists what must be done to make Section 16 real, and two clauses in it are worth memorising. Clause (g): books, other learning materials and appropriate assistive devices are to be provided free of cost to students with benchmark disabilities up to the age of eighteen. Clause (i): the curriculum and examination system are to be modified to meet the needs of students with disabilities, "such as extra time for completion of examination paper, facility of scribe or amanuensis, exemption from second and third language courses".

Above that sits Section 31, under which every child with benchmark disability between six and eighteen years has the right to free education. And Section 32 requires government and government-aided institutions of higher education to reserve not less than five per cent of seats for persons with benchmark disabilities, with a five-year upper age relaxation for admission.

What you want	The hook	What to say
Admission without refusal or a "we are not equipped" answer	Section 16(i)	"Section 16 requires admission without discrimination. What support does the school need from us?"
A seat near the board, sensory breaks, a quiet place for exams	Section 16(iii), reasonable accommodation	"I am requesting reasonable accommodation, not a special favour."
Extra time and a scribe in school tests	Section 17(i)	"The board already allows this in Class 10 and 12. I am asking for the same in school assessments."
Free books, learning material, assistive devices up to 18	Section 17(g)	"Under Section 17(g) these are to be provided free of cost. Who in the school handles that?"
A resource room instead of a paid shadow teacher	Section 16(iv), individualised support	"Individualised support can be a resource room. I would like to discuss that before a shadow teacher at Rs. 12,000 a month."
Transport, including for an attendant	Section 16(viii)	"If the school runs transport, Section 16(viii) covers the attendant of a child with high support needs."
A seat in a college course	Section 32	"Not less than five per cent of seats, and a five-year age relaxation at admission."

One honest nuance. Section 16 places the duty on government and local authorities to ensure that funded or recognised institutions comply. A recognised private school is within that description, but the practical enforcement route runs through the education department and the State Commissioner for Persons with Disabilities, not through an argument at the gate. Which is why the letter in the next chapter matters more than the speech.

CHAPTER 22

The accommodation request that works

Verbal requests get sympathetic nods and no change. One page, addressed to the principal, with the certificate attached and a copy marked to the district education officer, changes the temperature of the conversation, because it creates a record that someone has to answer.

The five parts of the letter

1. Who the child is: name, class, section, admission number.
2. What the child has, with the diagnosis and the certificate number, attached as an annexure.
3. What specifically you are asking for, numbered, in the language of the Act - not "understanding", but "extra 20 minutes in written assessments".
4. Why each ask is reasonable: one line each, from the therapist's report or the board's own circular.
5. A request for a written reply within fifteen days, and a request for a meeting.

The letter

To: The Principal, [School name]
Subject: Request for reasonable accommodation under Sections 16 and 17 of the Rights of Persons with Disabilities Act, 2016
Madam / Sir,
My child, [name], is a student of Class [x], admission number [y], and has been assessed with [diagnosis] by [clinician], whose report and the disability certificate (UDID [number]) are enclosed.
I request the following reasonable accommodations, each of which is supported by the enclosed report: (1) a seat in the first two rows; (2) an additional 20 minutes in written assessments; (3) exemption from copying from the board, with a photocopied note instead; (4) a short movement break once in each double period; (5) the school to record the child under the CWSN category at the Class IX / XI registration, so that board concessions are available.
Section 16 of the RPwD Act requires that children with disabilities be admitted without discrimination and provided reasonable accommodation according to their individual requirements, and Section 17(i) provides for modification of the examination system including extra time and a scribe.
I would be grateful for a written reply within fifteen days and for a meeting at your convenience. I am copying this to the District Education Officer for information.
Yours sincerely, [name, address, phone, date]. Enclosures: assessment report, disability certificate, UDID card copy, therapist's recommendation.

Send it by email and by hand, and keep the acknowledgement. If the school has no email, hand it to the office and photograph the receiving stamp on your copy. In the meeting, take one page of asks, not the whole file, and leave the file at home. Schools respond to a parent who is organised, not to a parent who is exhausted, and the letter is what makes you look organised even when you are not.

CHAPTER 23

Board exams: extra time, scribe, and the language exemption

CBSE's concessions for candidates with disabilities are set out in circular CBSE/COORD/112233/2019 dated 12 April 2019, which superseded earlier circulars and applies from the 2020 examinations onwards, and are consolidated in the board's CWSN exemption manual. The rules are specific enough to quote at a school that says it does not know.

Compensatory time

Length of paper	Extra time allowed
3 hours	60 minutes
2 hours 30 minutes	50 minutes
2 hours	40 minutes
1 hour 30 minutes	30 minutes

The extra time applies with or without a scribe, for the categories the circular allows, and a candidate does not have to take a scribe in order to get the time.

The scribe rules, in plain words

- The candidate may bring their own scribe or reader, or ask the examination centre to provide one.
- A candidate's own scribe must be one step below the candidate's qualification, with an undertaking in the circular's Annexure-B.
- A centre-provided scribe must not hold more than the minimum qualification for the exam, and must always be matriculate or above.
- The candidate may meet the scribe two days before the examination.
- One scribe per subject; different papers, especially languages, may have different scribes, and a scribe can be changed in an emergency.
- A candidate with a scribe sits in a separate room with a separate Assistant Superintendent.
- The scribe is free to the candidate; the centre pays the remuneration.
- A candidate who opts for a reader cannot also have a scribe. Choose with the child's teacher: a child who reads slowly but writes well needs a reader and time; a child who cannot get answers onto paper needs a scribe.

Subjects and languages in Class 10

For Class 10, candidates with disabilities as defined in the RPwD Act are exempt from the third language, may study one compulsory language instead of two, may choose subjects with flexibility from the board's listed groups, and may offer up to two skill subjects. The board's 2024 exemption manual adds the use of a computer or laptop without internet access and basic calculators for candidates registered under the CWSN category.

Which certificate is accepted? One issued by a government authority, by a recognised institution, or by a non-governmental organisation or practitioner registered with the Rehabilitation Council of India or the central or state government. A certificate issued by the competent authority anywhere in the country is accepted. The circular also carries a proforma, Annexure-A, for the physical

limitation on writing.

If your child is in an ICSE, IB or state board school, none of the above applies directly, but every board has an equivalent policy and every one of them is published. Ask the school's examination coordinator for the board's current concessions circular in writing, and ask in Class 8, not in Class 10.

CHAPTER 24

The Class 9 and Class 11 registration trap

This is the single most expensive administrative mistake in Indian school education for a child with a disability, and it is invisible until it is unfixable.

CBSE's concessions are available to a candidate only if the relevant disability category was entered during registration in Class IX or Class XI. The circular is explicit: only those students would be considered for grant of exemption or concession in whose respect the relevant category has been entered during registration. The registration happens through the school, in the List of Candidates process, months before the board exam - for the 2027 examinations, the list was signed in September 2026.

The consequence: a family that obtains a beautiful autism certificate in November of the board year has a certificate the board will not use for concessions, because the registration was filed without the CWSN category. The school will not always tell you, because the school does not know your child's certificate is coming.

1. In the first month of Class 9 (or Class 11), hand the school a copy of the certificate and a written request to register the child under the CWSN category.
2. Ask for written confirmation that the category has been entered, with the registration number. Keep the reply in the file.
3. If the certificate is still pending, ask the school in writing what it will do if the certificate arrives after the registration deadline, and get that answer in writing too.
4. In Class 9, also ask how the third-language requirement (R3) will be handled. CBSE's secondary curriculum for 2026-27 makes a third language compulsory from Class 9 and states that CWSN with certified specific learning disabilities or other specified conditions may be exempted as per the Examination Bye Laws. Ask the school to confirm in writing how it will apply that to your child, because the practice is not yet uniform.

CHAPTER 25

When the school says no

Schools say no for three reasons: they do not know the law, they do not want the work, or they genuinely believe they cannot cope. The response is the same in all three cases - escalate in writing, in order, without emotion, and keep going.

Step	Who	What you send
1	Class teacher	A one-page note with the three asks that matter most
2	Principal	The formal accommodation letter from chapter 22, copied to the DEO
3	School management / trust	A short email noting that the request is under the RPwD Act and that no reply has been received
4	District Education Officer / Block Education Officer	A complaint with the letter, the acknowledgement and the school's reply, or proof of no reply
5	State Commissioner for Persons with Disabilities	A written complaint under Sections 79 and 80 of the RPwD Act, asking for an inquiry and directions
6	Legal	A notice through an advocate; Section 89 provides for penalties for contravention of the Act

Two things not to do. Do not stop paying fees to force a conversation; it converts a rights dispute into a fee dispute and you will lose. And do not withdraw the child while the complaint is pending, because the school's first defence will be that you left voluntarily. Keep the child in school, keep the record clean, and let the paper do the arguing.

If the school is simply incapable, changing schools is a legitimate strategy, not a defeat. Ask three questions of any prospective school before you pay: how many children with disabilities are currently enrolled, does the school have a special educator or resource room, and has it ever registered a candidate under the CWSN category for board exams. The answers tell you in five minutes what a prospectus will never say.

What a school that is actually working looks like

The class teacher knows the diagnosis without being reminded. The timetable already has a movement break in it. The special educator has met your child before the first term ends. Test papers come home with extra time already marked, not after a fight. And the office can tell you, without looking it up, how many children with disabilities are enrolled this year. None of that is generosity; all of it is the Section 16 duty being done. When you find a school like this, say thank you out loud, because the staff there are usually paying for it out of their own patience.

The long game

Guardiansh
file that sav

CHAPTER 26

Guardianship: the paperwork nobody does until it is late

At eighteen your child becomes a legal adult. For a young person with autism, cerebral palsy, intellectual disability or multiple disabilities, that can mean a bank that will not let a parent operate an account, a hospital that will not accept a parent's consent, and a property transaction that cannot be signed by anyone at all. Guardianship is the instrument that fills that gap, and it is granted by a committee, not by a court.

Under Section 14 of the National Trust Act, 1999, the Local Level Committee - chaired by the District Magistrate or District Collector, with a representative of a registered organisation and a person with disability as members - is empowered to receive an application for appointment of a guardian and to appoint one. The application is made in Form A and the appointment is confirmed in Form B. The committee must meet at least once every three months.

Who can apply

- Both parents jointly, or a single parent where the other is absent due to death, divorce, legal separation, desertion or conviction.
- Siblings, including half and step siblings, jointly or singly, where both parents are absent.
- A relative, if none of the above applies.
- A registered organisation, if no family member applies - and the committee may direct one to apply for a destitute or abandoned person.

What the guardian must then do

Within six months of appointment, deliver to the appointing authority an inventory of the person's immovable property and all assets and movable property, with a statement of claims, debts and liabilities. Within three months of the close of every financial year, furnish an account of the property and assets in charge, sums received and disbursed, and the balance remaining. This is not a formality; it is the accountability that makes the appointment meaningful.

Guardianship gives you	Guardianship does not give you
Authority to act on personal care: food, shelter, health, education, training, protection from exploitation	The right to override the person's own expressed wishes in every matter
Standing to deal with banks, hospitals and government offices on the person's behalf	Ownership of the person's property
A documented, official status that institutions recognise	A substitute for a will or a trust
A mechanism the committee can monitor and, if needed, remove	Permanence without review

Two current notes. The National Trust published a clarification in June 2026 on the appointment of legal guardians for persons with mental retardation, cerebral palsy, autism and multiple disabilities under the RPwD Act and the National Trust Act, so ask the committee for its current practice rather than relying on an old checklists from a forum. And the RPwD Act itself, in Sections 13 and 14,

provides for legal capacity and for guardianship, including limited guardianship where a person can manage some decisions with support - which is why the guardianship order should say what it covers, not simply that a guardian exists.

File this at seventeen, not at twenty

Documents to assemble: the UDID card, the disability certificate, the child's Aadhaar, the parents' Aadhaar, proof of residence, a medical report stating that the person cannot manage his or her own affairs, the family tree, and Form A. Take two copies of everything. The committee sits quarterly, and a missing document costs a quarter.

CHAPTER 27

Adult life: day care, group homes and respite

The National Trust runs five welfare schemes, and they exist precisely because the school years end. Knowing the names matters, because the organisations that run them are listed on the Trust's website and the ones near you are the ones you can visit.

Scheme	For whom	What it provides
DISHA	Children 0 to 10 with the four National Trust disabilities	Early intervention and school readiness at a Disha centre, at least four hours a day, with a special educator or early intervention therapist, a physiotherapist or occupational therapist, a counsellor, caregivers and ayas
VIKAAS	Children and young adults	Day care services at a centre, with age-specific activities
SAMARTH	Orphans, abandoned persons, families in crisis, and persons with disabilities from BPL and lower-income families	Respite care and group home facilities, giving families planned relief
GHARAUNDA	Adults	An assured home and minimum quality of care for life, with basic medical care, pre-vocational and vocational activities
NIRAMAYA	All ages with the four disabilities	The Rs. 1 lakh health insurance from chapter 15

These are delivered through registered organisations, not by the government directly. So the practical step is to find the registered organisations in your state, visit two of them without your child first, and ask three questions: how many people live here, what happens when a resident's parents die, and who inspects. The answers separate a home from a warehouse.

What to start at sixteen

- A skills assessment: what can the young person do alone, with prompting, and not at all. Write it down annually; it is the basis of every vocational plan.
- A vocational track: DDRC skill training, National Institute programmes, or a family-run micro-business financed through NDFDC.
- The guardianship file, started at seventeen.
- A written care plan, one page, that names the person who takes over if something happens to you.

CHAPTER 28

Work: the four per cent, and the jobs it opens

Section 34 of the RPwD Act requires every appropriate government to appoint not less than four per cent of the total number of vacancies in the cadre strength in each group of posts for persons with benchmark disabilities. The four per cent is broken down by category: one per cent each for blindness and low vision; deaf and hard of hearing; locomotor disability including cerebral palsy, leprosy cured, dwarfism, acid attack victims and muscular dystrophy; and one per cent for autism, intellectual disability, specific learning disability and mental illness, with multiple disabilities drawn from these groups.

Two things that surprise families. Speech and language disability, although a specified disability, does not have a separate reservation category - the department confirmed this in Parliament. And the reservation is in posts identified for each disability, so the vacancy list, not the advertisement, is what decides.

The rest of the employment chapter

- Section 20: no discrimination in employment on the ground of disability, including in promotion and conditions of service.
- Section 21: every establishment with twenty or more employees must frame an equal opportunity policy.
- Section 23: a grievance redressal officer must be appointed.
- Section 35: incentives to private-sector employers who employ persons with disabilities.
- Section 36: special employment exchanges for persons with disabilities.
- Section 19: vocational training and self-employment schemes, which is where NDFDC and DDRC skill training come in.

For a young person who will not sit in a competitive exam, the realistic routes are three: a government post through the identified-vacancy list, a private employer with an equal opportunity policy and a disability hiring programme, or self-employment financed at five to nine per cent. All three need the same three documents - UDID, certificate, and a skills record - so the work of the last ten chapters is also the work of the first job.

CHAPTER 29

The family file: wills, siblings and records

This is the chapter that gets skipped, and it is the one whose absence is felt by the sibling who is left holding everything at forty-five with no idea what the plan was.

What belongs in the file

Document	Why it matters	Where the original lives
UDID card and disability certificate	Every scheme, every concession, every admission	Home file; scan in cloud
Guardianship order (Form B)	Banks, hospitals, property, consent	Home file plus a second copy with the advocate
Assessment reports, all of them	Reassessment, appeals, school, board concessions	Home file; scan every year
School records and board registration	Concessions and college admission	Home file
Insurance policies, including Niramaya	Claims and renewals	Home file plus a note of policy numbers
Bank and investment details	Continuity for the person after you	Sealed envelope with the executor
Will, naming a guardian and a trustee	The whole point	With the advocate and the executor
Letter of intent	The unwritten knowledge: routines, doctors, what soothes him	Home file, reviewed yearly

Three decisions to make deliberately

- **A will that names a guardian and, where the estate justifies it, a trust.** Leaving assets to a person who cannot manage them, without a trust, is the most common way a good intention becomes a family fight. Have this drafted by a lawyer who has done it before; the cost of the drafting is trivial next to the cost of the alternative.
- **Insurance on the earning parent, not on the child.** The financial risk is the loss of the person who pays for therapy, not the child's own mortality. Term insurance on the earning parent, with a nomination that is actually updated, is the cheapest protection available.
- **A sibling briefing.** One conversation a year, with the file open, is worth more than a document nobody has read. Say out loud what you want, what the money is for, and who decides.

CHAPTER 30

Your first twelve months, month by month

Start from wherever you are. Month one is the month you begin, not January.

Month	What to do	Why now
1	Book the assessment; start the behaviour diary; open the ring file	Everything downstream needs a typed report
1	Call the district hospital and ask about DEIC and the UDID board dates	Queues are measured in months
2	Attend the assessment; get the typed report with registration number	The report is the key
2	Walk into the DEIC with the file and ask for a therapy plan	Free therapy starts the day you ask
3	Apply for the UDID on the portal; save the enrolment number	The clock starts at application

Month	What to do	Why now
4	Attend the medical board; ask what percentage was recorded	You cannot appeal what you do not know
4	Enrol in Niramaya if the disability is one of the four	Rs. 1 lakh of cover for Rs. 500
5	Apply for ADIP through ARJUN or a PMDK for any device prescribed	Three-year cycle; start early
6	Submit the school accommodation letter; ask for CWSN registration confirmation	Class 9 or 11 is when it must be recorded
7	Review the therapy budget; drop what has not moved a goal	Money is finite and so is patience
8	Check the tax position; collect the certificate for the deduction	A flat deduction needs one document
9	Map the scholarship cycle for next year; get the income certificate made	The portal opens in June and closes in October
10	File the Section 59 appeal if the percentage is wrong	Ninety days from issue
11	Visit two National Trust registered organisations for the adult years	You are planning a decade, not a term
12	Review the file; scan everything; write the one-page care plan	Continuity is the real inheritance

If you do nothing else this year, do three things: get the typed report, apply for the UDID, and put the accommodation request in writing. Those three produce more change than any other three actions available to an Indian parent, and all three are free.

The whole book in five lines

One: a typed diagnosis, with the tool and the score, is the document every door in this country asks for. Two: the UDID card is free and slow, so apply in the month you get the report, not the month you need it. Three: free therapy already exists in your district, and asking for it is not charity. Four: Rs. 1 lakh of insurance, a Rs. 1.25 lakh deduction and a tuition-paying scholarship are yours by rule, not by favour. Five: the school has a legal duty to admit your child and accommodate the disability, so put every request in writing and keep the acknowledgement.

Appendices

The checkl
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The master document checklist

One page. Photograph it, or print it and stick it inside the ring file. Tick as you go.

Document	Have	Note
Typed diagnosis report with stamp and registration number		The foundation
Child's Aadhaar		Or the enrolment slip
Parent's Aadhaar and address proof		Address decides hospital and post
UDID enrolment number and acknowledgement		Two copies
UDID card and e-certificate		Download from the portal; save in DigiLocker
Audiology report		If speech is the presenting concern
Psychological or developmental assessment with scores		Needed for appeals and scholarships
Behaviour diary, four weeks		The most persuasive page you own
Therapy plan with frequency and goals		From the DEIC or the private clinic
School accommodation letter and its acknowledgement		Dated, signed, copied to the DEO
CWSN registration confirmation		Class 9 or 11, in writing
Niramaya e-card		Note the cover dates
Income certificate from the revenue office		For scholarships and ADIP
Bills and receipts, filed by month		For Niramaya claims and tax
Guardianship order (Form B), from age 18		Filed at 17
Will and letter of intent		Reviewed yearly

The four ladders, on one page

Problem	Ladder
UDID stuck	Hospital UDID cell - CMO or District Welfare Officer - State UDID coordinator - District Collector - State Commissioner for PwD - CPGRAMS
Wrong disability percentage	Appeal on the UDID portal within 90 days (Section 59) - one appeal only - if no decision in three months, apply afresh
School refuses accommodation	Class teacher - principal in writing - school management - DEO or BEO - State Commissioner for PwD - legal notice
Insurer refuses or loads the premium	Written refusal - insurer grievance officer - IRDAI Bima Bharosa - insurance ombudsman for the state

The one sentence that opens most doors

"I am not asking for a favour. I am asking for what Section ___ of the Rights of Persons with Disabilities Act provides, and I would like your reply in writing." Say it once, calmly, and then send the letter. It works because the person across the desk knows the Act exists and does not want to be the officer who ignored it in writing.

Glossary

Term	What it means
ADIP	Assistance to Persons with Disabilities for Purchase or Fitting of Aids and Appliances: the central scheme that pays for devices, kits and implants
ALIMCO	Artificial Limbs Manufacturing Corporation of India, which manufactures and supplies devices
ARJUN	The department's online portal for ADIP and the Rashtriya Vayoshri Yojana
AYJNISHD	Ali Yavar Jung National Institute of Speech and Hearing Disabilities, the nodal agency for cochlear implants
Benchmark disability	A certified disability of not less than 40 per cent, which is the threshold for most benefits
CRC	Composite Regional Centre: the regional arm of a National Institute
CWSN	Children With Special Needs: the category schools and boards use in registration
DDRC	District Disability Rehabilitation Centre: district-level therapy, devices and UDID help
DEIC	District Early Intervention Centre: free multidisciplinary assessment and therapy, birth to 18
DEPwD	Department of Empowerment of Persons with Disabilities, Ministry of Social Justice and Empowerment
ISAA	Indian Scale for Assessment of Autism: the tool used to convert autism severity into a disability percentage
INDT-ASD	The DSM-5-based, AIIMS-modified INCLEN diagnostic tool for autism spectrum disorder
LLC	Local Level Committee, chaired by the District Magistrate or Collector, which appoints legal guardians under the National Trust Act
NDFDC	National Divyangjan Finance and Development Corporation: concessional loans
Niramaya	The National Trust's Rs. 1 lakh health insurance scheme for four disability groups
PMDK	Pradhan Mantri Divyasha Kendra: walk-in centre for ADIP devices
RBSK	Rashtriya Bal Swasthya Karyakram: the national child screening programme
Reasonable accommodation	Changes to the environment, curriculum or process that let a person with disability participate on an equal basis
RPwD Act, 2016	The Rights of Persons with Disabilities Act, which recognises 21 disabilities and sets out rights, quotas and remedies
TLM kit	Teaching and Learning Material kit, available under ADIP for children with intellectual and developmental disabilities
UDID	Unique Disability ID: the card and certificate issued on swavlambancard.gov.in

Sources

Every figure, section number and scheme rule in this book was checked against the following, in November 2026. Where a cost is quoted, it is a reported cost from a named publication and is described as such in the text. Where two official documents disagree - the ADIP waiting period for children is the one flagged in this book - both versions are stated and you are told to confirm at the counter. That is deliberate: a guide that pretends certainty about a moving scheme is worse than no guide.

Source	Used for
Rights of Persons with Disabilities Act, 2016, India Code	Sections 3, 16, 17, 31, 32, 34, 58, 59, 79, 80; the 21 specified disabilities; the benchmark definition
DEPwD UDID scheme pages and portal SOPs (July and October 2024); Gazette notification dated 12 March 2024 (S.O. 1272)	UDID application, fees, documents, appeal within 90 days, rejected applications; autism assessment by INDT-ASD and ISAA, the under-six rule, validity
DEPwD, ADIP scheme page and revised ADIP scheme (26 September 2024)	Eligibility, income slabs, device amounts, cochlear implant, TLM kits, ARJUN and PMDK
DEPwD, DDRC page; DEPwD Annual Report 2025-26	DDRC services, one per district, therapeutic services
National Health Mission, RBSK 2.0 Operational Guidelines 2026	Screening platforms, ages, frequency, DEIC staffing, free services, referral card
National Trust: Niramaya scheme pages and policy document	Cover, benefit chart, premium, renewal, claim rules, helpline
National Trust: guardianship and Local Level Committee pages, clarification of June 2026, and schemes pages	Guardianship under Section 14, Forms A and B, duties of a guardian; DISHA, VIKAAS, SAMARTH and GHARAUNDA
DEPwD scholarship guidelines (25 September 2024) and National Scholarship Portal	Pre-matric, post-matric and Top Class amounts, income ceilings, slots
NDFDC credit guidelines and FAQs	Loan slabs, interest rates, eligibility, channelizing agencies
DEPwD free coaching scheme and National Fellowship scheme pages	Fees, stipend, allowances, fellowship rates
CBSE circular CBSE/COORD/112233/2019 and CWSN exemption manual 2024	Extra time, scribe rules, language exemption, calculator and computer, registration condition
UGC directions of August 2026 on RPwD compliance	Section 32 compliance, five per cent reservation, annual reporting
IRDAI circular IRDAI/HLT/CIR/MISC/58/2/2023 dated 27 February 2023; insurance ombudsman order reported October 2026; Supreme Court proceedings reported September 2026	PwD product obligation, underwriting policy, no denial for disability; current position on premiums, co-payment and data on policies issued
Income Tax Department comparison utility; published mappings of Chapter VI-A to Chapter VIII	Sections 127 and 154 (old 80DD and 80U)
The Times of India, Patna, April and August 2026; The Core, April 2026; Scroll, October 2026; India Today, April 2026	Reported therapy costs, monthly and annual spend, UDID processing times, diagnostic delays

A last word

Three things this book cannot do for you, said plainly, because pretending otherwise would be a worse kindness.

- It cannot make the system fast. A disability board meets when it meets, and a certificate takes the time it takes. What you can control is when the clock starts, and starting it this month instead of next year is the whole game.
- It cannot tell you what your child will be able to do at twenty-five. Nobody can, and anyone who offers a prediction at age four is selling something. The evidence is consistent on one point only: early, consistent intervention changes outcomes, and the amount of it matters more than the brand of it.
- It cannot carry the emotional weight of this. That is not a paperwork problem and no checklist touches it. If you are the parent doing all of this alone, find one other parent who has been through the disability board in your district, and keep their number. The practical help is real, and so is having someone to call at eleven at night.

If you only have forty-eight hours

1. Write the first line of the behaviour diary tonight.
2. Create the login on swavlambancard.gov.in and save the password where your spouse can find it.
3. Send one email to the school asking for a meeting, and cc the class teacher.
4. Put a reminder in your phone for the first Monday of next month: call the DEIC and ask for a slot.

Everything else in these pages can wait a week. Those four cannot, because they are the ones that start clocks, and clocks are the only thing in this system that money cannot buy back.

One more thing

Keep this book in the ring file, not on the shelf. The chapters you will re-read are chapter 23 before board exams, chapter 18 in June when the scholarship portal opens, and chapter 26 the year your child turns seventeen. Write the dates in the front cover now.